

Supplemental Table 1. Institutional guidelines for the use of *ex-situ* normothermic machine perfusion (NMP). Note that while these guidelines may help surgeons decide on the use of NMP, the decision is ultimately up to the primary transplant surgeon regarding its' use on a case-by-case basis. Note that we routinely perfuse all DCD livers, where all other factors are under consideration on case-by-case basis.

Donor Factors	Recipient Factors	Logistical Factors
All Donation after circulatory Death (DCD) liver	Recipient with significant cardiac** or pulmonary diseases##	Expected CIT > 8 hours
Donor Age > 60 years	Hemodynamic instability	Personnel or other logistical limitations
Macrosteatosis of > 30%	Combined open heart surgery and LT (CABG, Valve repair, pericardiectomy)	
Open offer organ (orphan livers)	Combined heart + liver or lung + liver transplantation	
Liver declined during or after organ recovery		
Peak Liver Enzymes (AST/ALT) > 1,000 IU/L and/or Peak total Bilirubin > 2 mg/dL		

** Coronary Artery Disease, severe valvular disease. ## COPD, pulmonary hypertension